

Multi-country outbreak of cholera



Epidemiological Update n. 34 published 21 February 2026

Cases – 16 912
Since Jan. 2026

Deaths – 182
Since Jan. 2026

Countries affected – 19
Since Jan. 2026

Population at risk
1 billion

Global risk –
Very high

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Overview

Data as of 25 January 2026

- In January 2026 (epidemiological weeks 1 to 4), a total of 16 912 new cholera and acute watery diarrhoea (AWD) cases were reported from 19 countries, territories, and areas (hereafter countries) across three WHO regions, showing a 6% decrease from the previous month. The African Region registered the highest number of cases, followed by the Eastern Mediterranean Region, and the South-East Asia Region. This period also saw 182 cholera-related deaths globally, highlighting a 20% increase from the previous month. No outbreaks were reported in the Western Pacific Region or Region of the Americas during this time.
- Cholera cases and deaths in January 2026 were 57% and 54% lower than in January 2025 (39 434 cases and 398 deaths across 23 countries).
- In recent years, conflict, mass displacement, disasters from natural hazards, and climate change have intensified outbreaks, particularly in rural and flood-affected areas, where poor infrastructure and limited healthcare access delay treatment. These cross-border factors have made cholera outbreaks increasingly complex and harder to control.
- The data presented here should be interpreted with caution. Potential underreporting and reporting delays may affect timeliness and accuracy, while variations in surveillance systems, standard case definitions, and laboratory capacities can limit direct comparability among countries. These factors also influence the global case fatality rate (CFR), requiring careful examination. Unless otherwise specified, the term 'cholera cases' includes both suspected and confirmed cases. Data in this report may be adjusted retrospectively as more information becomes available. For the latest data, please consult WHO's [Global Cholera and AWD Dashboard](#).

Global epidemiological update

In January 2026, (epidemiological weeks 1 to 4), a total of 16 912 new cholera and AWD cases were reported from 19 countries across three WHO regions, representing a 6% decrease from the previous month and a 57% decrease compared to January 2025 (39 434 cases across 23 countries). The global decline was largely driven by significant reductions in the number of cases reported across the African Region and Eastern Mediterranean Region.

In January 2026, the African Region reported the highest number of cases (9782 cases; 13 countries), followed by the Eastern Mediterranean Region (7069 cases; five countries) and the South-East Asia Region (61 cases; one country). No cases were reported from the Western Pacific Region or the Region of the Americas.

During the same period, 182 cholera-related deaths were reported globally. While this marks a 20% increase compared to December 2025, it remains 54% lower than the mortality burden of January 2025 (398 deaths). In January 2026, the highest number of deaths was recorded in the African Region (181 deaths; 9 countries), followed by the Eastern Mediterranean Region (one death; one country). No deaths were reported from the South-East Asia Region, the Western Pacific Region, or the Region of the Americas.

Figure 1. Cholera and acute watery diarrhoea (AWD) cases per 100 000, 1 to 25 January 2026

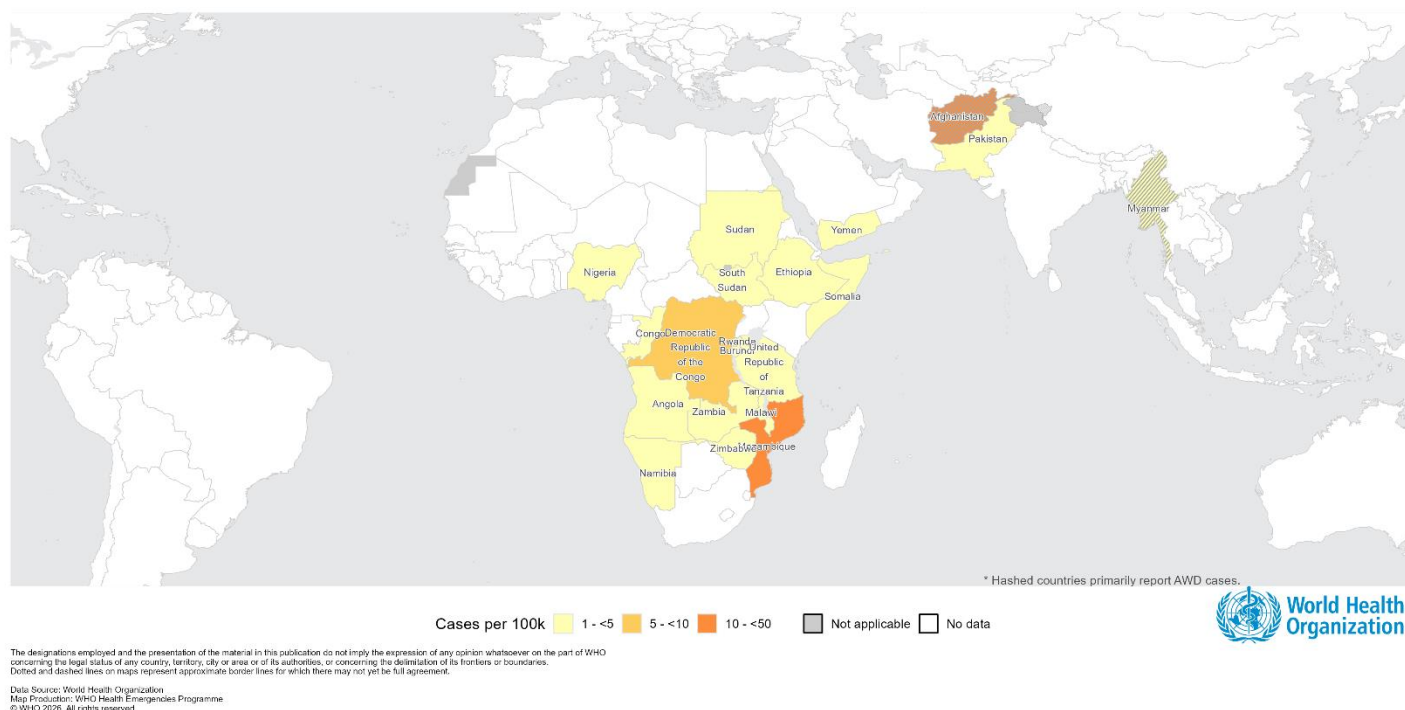


Table 1. Reported cholera and AWD cases and deaths by WHO Region, as of 25 January 2026

Region	Country, territory, area	1 to 25 January 2026					
		Cases	Deaths	CFR (%)	Cases per 100 000	Monthly cases % change	Monthly deaths % change
African Region	Angola	143	6	4.2	<0.1	-83	-60
	Burundi	299	0	0.0	2	-63	-100
	Democratic Republic of the Congo	6 543	148	2.3	5	39	66
	Ethiopia	15	0	0.0	<0.1	-74	-
	Malawi	411	6	1.5	2	414	-
	Mozambique	1 287	8	0.6	4	32	-60
	Namibia	42	0	0.0	1	-2	-
	Nigeria	371	3	0.8	<0.1	-7	-70
	Rwanda	16	0	0.0	<0.1	-	-
	South Sudan	455	6	1.3	4	-12	100
	United Republic of Tanzania	113	2	1.8	<0.1	565	-
	Zambia	81	1	1.2	<0.1	80	-
	Zimbabwe	6	1	16.7	<0.1	-	-
Eastern Mediterranean Region	Afghanistan***	5 391	0	0.0	16	-18	-100
	Pakistan [§]	493	0	0.0	<0.1	-46	-
	Somalia	233	0	0.0	1	-25	-
	Sudan	9	0	0.0	<0.1	-86	-
	Yemen [¶]	943	1	0.1	3	-38	-50
South-East Asia Region	Myanmar***	61	0	0.0	<0.1	-9	-

* Case and death numbers presented are not directly comparable due to differences in case definitions, reporting systems, and general underreporting. All data are subject to verification and change due to data availability and accessibility. Respective figures and numbers will be updated as more information becomes available. The data in Table 1 includes suspected, rapid diagnostic test (RDT) positive, and culture-confirmed cholera cases. As multiple countries report only total data on deaths, the reported CFR is calculated throughout based on the total number of deaths reported. The Global Task Force on Cholera Control (GTFCC) recommends that CFR be calculated using only facility deaths, with the number of community deaths reported separately.

** Any missing data in this report does not imply the absence of cholera or AWD cases or deaths in the respective country. The data presented in this report are based on the latest available information and may not reflect the current situation.

*** Afghanistan and Myanmar report AWD cases.

[§] The reported number of suspected cholera and AWD cases is based on the available [Public Health Bulletin published by the National Institute of Health of Pakistan](#).

[¶] Includes all reported suspected cholera and AWD cases from Yemen.

WHO regional overviews

African Region

In January 2026, the African Region reported 9782 new cholera cases across 13 countries, marking a 15% increase compared with December 2025. The highest number of cases were reported from the Democratic Republic of the Congo (6543), Mozambique (1287), and South Sudan (455). A total of 181 cholera-related deaths were reported, representing a 27% increase compared with the previous month. The highest numbers of deaths were reported from the Democratic Republic of the Congo (148); Mozambique (8); and Angola, Malawi, and South Sudan (6 each).

Eastern Mediterranean Region

In January 2026, the Eastern Mediterranean Region reported 7069 new cholera and AWD cases across five countries, marking a 25% decrease compared with December 2025. The highest number of cases were reported from Afghanistan (5391), Yemen (943), and Pakistan (493). One cholera-related death was reported in Yemen, representing an 80% decrease compared with the previous month.

South-East Asia Region

In January 2026, the South-East Asia Region reported 61 new AWD cases in Myanmar, marking a 22% decrease compared with December 2025. No cholera or AWD-related deaths were reported during this period.

Figure 2. Global cholera and AWD cases by week, 1 January 2024 to 25 January 2026

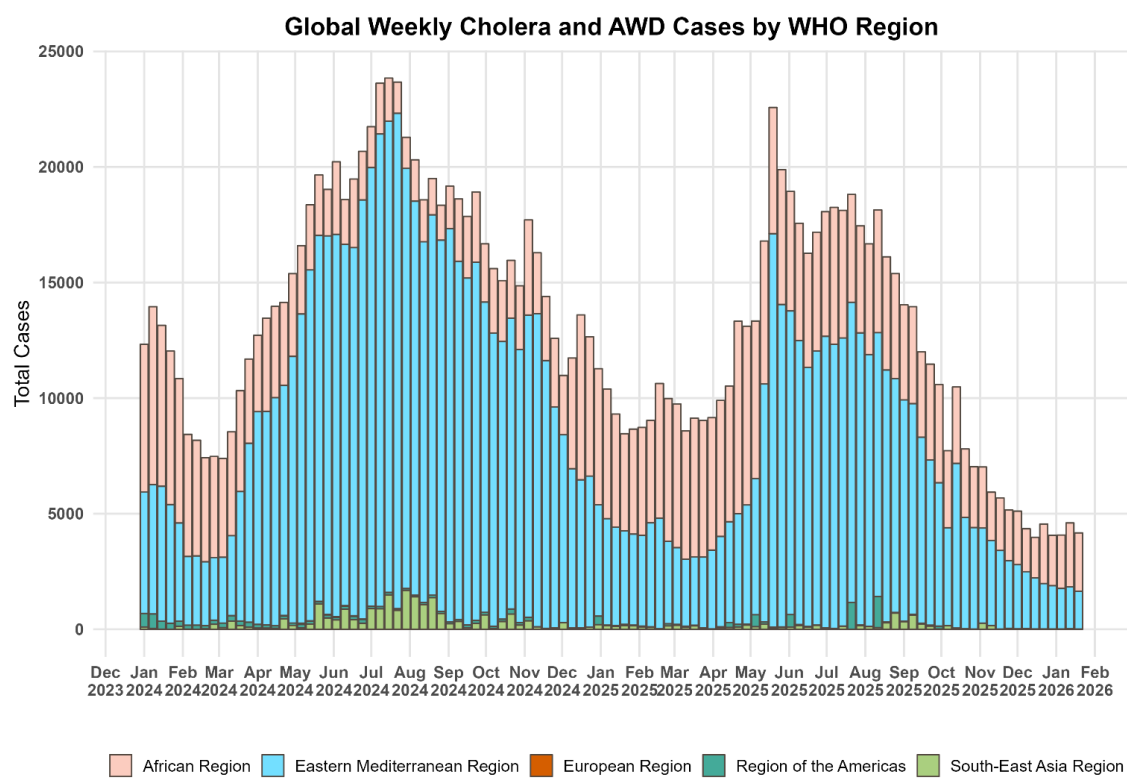
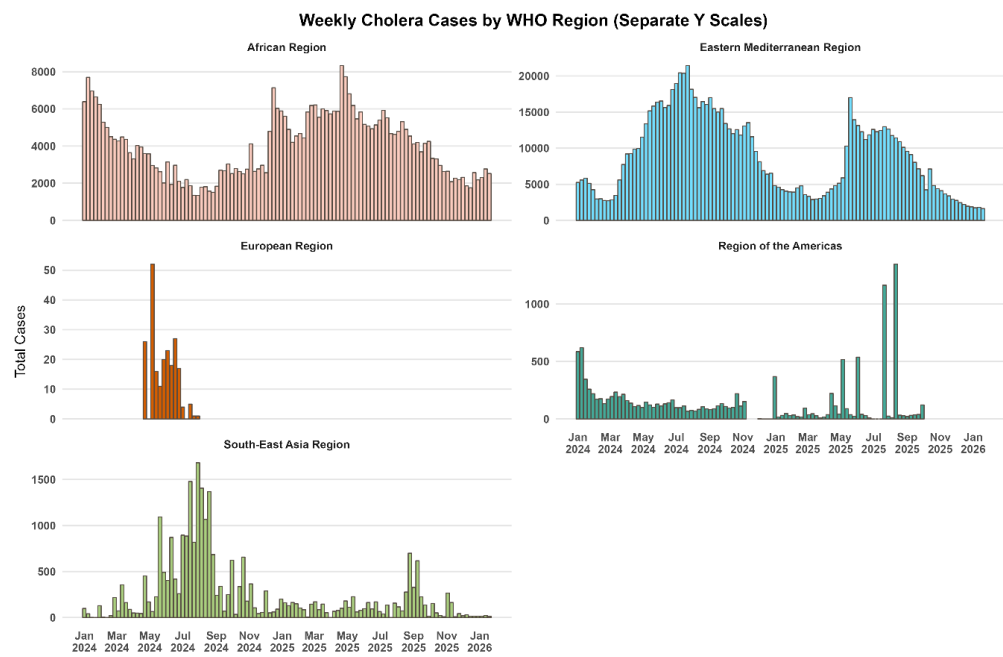


Figure 3. Cholera and AWD cases by WHO Region (separate Y scales), 1 January 2024 to 25 January 2026



*The epi curve of the Western Pacific Region is not included due to limited available weekly data.

**Spikes in the Region of the Americas are due to batch reporting.

Focus on selected subregions and countries

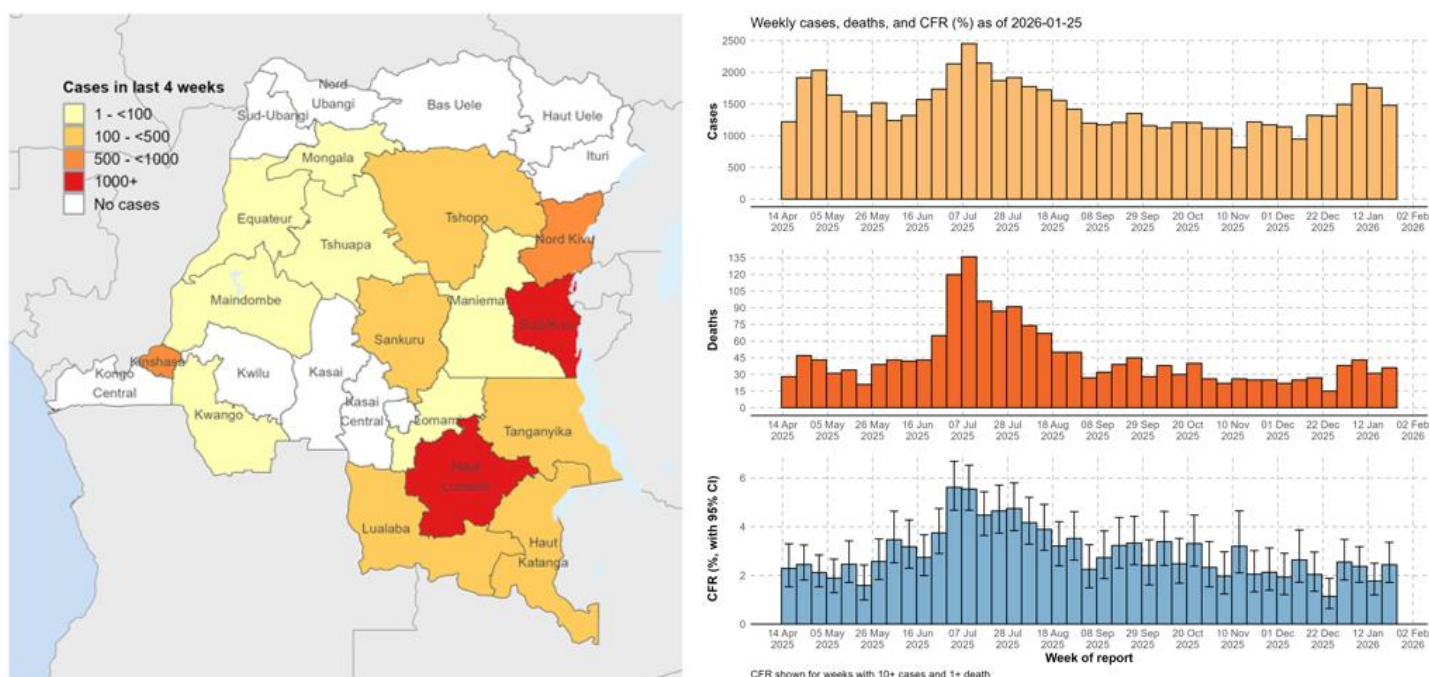
Democratic Republic of the Congo

In January 2026, the Democratic Republic of the Congo reported 6543 new cholera cases and 148 associated deaths (CFR: 2.3%). This represents a 39% increase in cases and a 66% increase in deaths compared with December 2025.

In the last four weeks, the highest proportions of cases were reported from Sud-Kivu (35%), Haut Lomami (16%), Nord-Kivu (14%), Kinshasa (9%), Haut Katanga (7%), and Tanganyika (7%). Most cases and deaths were reported from eastern provinces, where insecurity continues to restrict humanitarian access to affected communities.

Additional cases have been reported in fishing communities along the Congo River and around Lake Tanganyika, as well as in Sankuru province, where high population mobility and limited access to safe water and sanitation increase the risk of transmission. The increasing number of cases in Kinshasa heightens the risk of further spread to other provinces, particularly given frequent population movement along the Congo River.

Figure 4: Democratic Republic of the Congo: Distribution of cumulative cases by province (left) and weekly case, death, and CFR trends (right), as of 25 January 2026



The designations employed and the presentation of the material in this publication do not imply the expression of any opinion whatsoever on the part of WHO concerning the legal status of any country, territory, city, or area or of its authorities, or concerning the delimitation of its frontiers or boundaries. Dotted and dashed lines on maps represent approximate border lines for which there may not yet be full agreement.

Data Source: World Health Organization, Ministry of Health
Democratic Republic of the Congo
Map Production: World Health Organization

 **World Health Organization**
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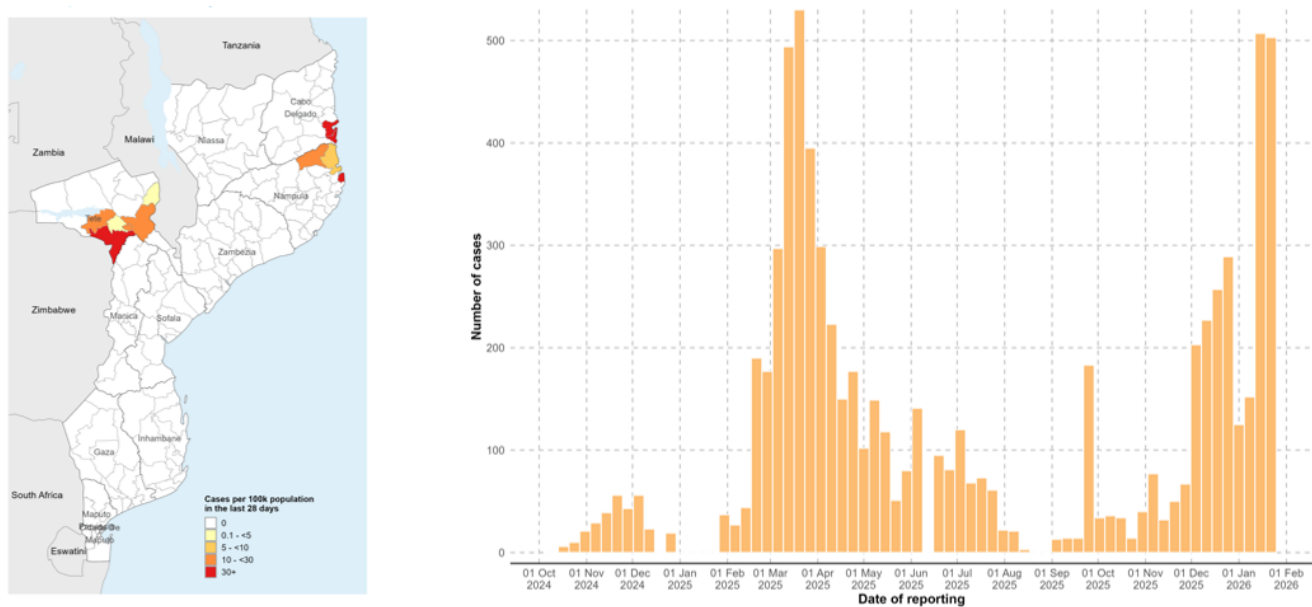
Mozambique

In January 2026, Mozambique reported 1287 new cholera cases and 8 associated deaths (CFR: 0.6%). This represents a 32% increase in cases and a 60% decrease in deaths compared with December 2025.

An increase in cases has been reported since late 2025. In the last four weeks, geographic spread was observed across three provinces: Nampula, Tete, and Cabo Delgado. Since December 2025, heavy rains and flooding have further increased transmission risk, particularly in areas with limited access to safe water and sanitation.

In Nampula province, a high number of cases was reported in Nacala Porto (673 cases) during January 2026. New cases have also been identified in parts of Tete Province, including communities where water, sanitation and hygiene conditions are poor and access to health facilities is limited. Population movement in cross-border areas may increase the risk of further spread to other countries.

Figure 5: Mozambique: Distribution of cumulative cases per 100 000 population by province in the last 28 days (left) and weekly case trend (right), as of 25 January 2026.



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Data Source: World Health Organization, Federal Ministry of Health Sudan
Map Production: World Health Organization

Annex 1. Data, table, and figure notes

Caution must be taken when interpreting all data presented. Differences are to be expected between information products published by WHO, national public health authorities, and other sources using different inclusion criteria and different data cut-off times. While steps are taken to ensure accuracy and reliability, all data are subject to continuous verification and change. Case detection, case definitions, laboratory testing strategies, reporting practice, and lag times differ across countries/territories/areas. These factors, amongst others, influence the counts presented, with variable underestimation of the true case and death counts, and variable delays in reflecting these data at the global level.

'Countries' may refer to countries, territories, areas, or other jurisdictions of similar status. The designations employed, and the presentation of these materials do not imply the expression of any opinion whatsoever on the part of WHO concerning the legal status of any country, territory, or area or of its authorities, or concerning the delimitation of its frontiers or boundaries. Dotted and dashed lines on maps represent approximate border lines for which there may not yet be full agreement. Countries, territories, and areas are arranged under the administering WHO region. The mention of specific companies or of certain manufacturers' products does not imply that they are endorsed or recommended by WHO in preference to others of a similar nature that are not mentioned. Errors and omissions excepted; the names of proprietary products are distinguished by initial capital letters.

Annex 2. Technical guidance and other resources

General

- [Cholera fact sheet](#)
- [Ending Cholera, A Global Roadmap To 2030](#)
- [Global cholera strategic preparedness, readiness, and response plan 2023/24](#)
- [WHO's Call for urgent and collective action to fight cholera](#)
- [Disease outbreak news Cholera – Multi-country with a focus on countries experiencing current surges](#)
- [Global Task Force on Cholera Control \(GTFCC\)](#)
- [AFRO Weekly outbreaks and emergency bulletin](#)
- [WHO AFRO Cholera Dashboard](#)
- [Cholera upsurge \(2021-present\) web page](#)

Training

- GTFCC Laboratory training on [Sample collection and testing with Rapid Diagnostic Tests for cholera for health care workers](#) available in English, French, Portuguese and Arabic.
- GTFCC Laboratory [job aids and fact sheets](#) available in English, French, Portuguese and Arabic.
- GTFCC Cholera surveillance for health authorities as well as on cholera surveillance health care workers. These courses are available in [English and French](#).
- Countries are encouraged to periodically self-assess their cholera surveillance systems using the [GTFCC method to assess cholera surveillance](#) to identify key activities for strengthening surveillance in line with GTFCC recommendations.

Technical guidance

- [GTFCC fixed ORP interim guidance and planning](#)
- [Public health surveillance for cholera, Guidance document, 2024 including tools and job aids](#). These recommendations are available in English, French, Arabic and Portuguese.
- [Recommendations for reporting cholera to the regional and global levels, 2025](#)
GTFCC [updated recommendations for cholera reporting to the regional and global levels](#). This comes along with a [reporting template](#) in Excel. It is available in English, [Arabic](#), [French](#) and [Portuguese](#).